

POLICY BRIEF

LANGUAGE ACCESS AND HEALTHCARE NAVIGATION

STRENGTHENING LANGUAGE ACCESS AND HEALTHCARE NAVIGATION FOR IMMIGRANT COMMUNITIES

A practical federal strategy to turn existing civil-rights protections into reliable, patient-centered access.

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- **Language barriers can compromise comprehension, informed consent, patient experience, and safety.**
- **Federal standards already require meaningful language access, but providers need stronger financing, workforce capacity, and accountability.**
- **HHS should jointly fund qualified interpretation and bilingual community health workers at safety-net providers**

What is the issue?

Immigrant communities with limited English proficiency may encounter overlapping barriers related to communication, insurance, transportation, employment, housing, and healthcare navigation.

Language proficiency and immigration status are distinct; however, either can shape whether a patient obtains timely, understandable, and appropriate care. Language discordance can affect comprehension, informed consent, adherence, satisfaction, and quality of care (Flores, 2006; Jacobs et al., 2006).

Undocumented immigrants may face additional financial, eligibility, discrimination, and fear-related barriers that increase reliance on safety-net services (Hacker et al., 2015).

Social conditions intensify these challenges. Jobs without employer-sponsored coverage, inflexible schedules, unstable housing, transportation limitations, and unfamiliarity with the health system can delay preventive and primary care.

THE CORE POLICY PROBLEM

Patients may have a legal right to language assistance while their local providers still lack the resources and workflows to deliver it consistently.

Why is this important?

PATIENT SAFETY

Communication failures can contribute to misunderstandings about diagnoses, medications, follow-up instructions, and consent.

TIMELY ACCESS

Navigation and affordability barriers can shift care away from prevention and toward later, more urgent treatment.

EQUITY

Patients should receive understandable information and meaningful access regardless of the language they speak.

SYSTEM PERFORMANCE

Reliable language services and navigation can improve referral completion, continuity, patient experience, and staff workflows.

Existing protections

Title VI and Section 1557 require many federally funded or covered health programs to provide meaningful language access without charging the patient (OCR, 2025). The National CLAS Standards call for free language assistance, timely notice, competent interpreters, and accessible translated materials (OMH, n.d.). The remaining challenge is consistent implementation.

What should policymakers do?

HHS should establish a joint CMS–OCR Language Access and Navigation Initiative. The initiative should prioritize federally qualified health centers, rural providers, community health centers, and other safety-net organizations serving substantial populations with limited English proficiency.

RECOMMENDED ACTION

Pair sustainable financing for qualified interpretation with bilingual community health workers, standardized training, clear patient notification, and annual performance reporting.

Five implementation actions

1. Create grant and reimbursement pathways for qualified in-person, telephone, and video interpretation.
2. Fund bilingual community health workers to provide education, navigation, and closed-loop referrals.
3. Standardize staff training and notices explaining that qualified language assistance is free.
4. Provide small and rural organizations with technical assistance for needs assessment, translated materials, vendor selection, and workflow design.
5. Require annual reporting by preferred language while minimizing immigration-status data collection and protecting patient privacy

Who should lead?

CMS should lead financing and delivery-system implementation. OCR should lead civil-rights guidance and accountability. The Office of Minority Health should provide CLAS-aligned technical assistance. State Medicaid agencies, health centers, interpreter organizations, and community-based organizations should participate in pilot design.



How should progress be measured?

- Preferred language documented in the health record
- Qualified interpreter used when language assistance is needed
- Staff completing language-access training
- Patient-reported comprehension and experience
- Referral closure and preventive-care follow-through
- Wait times and missed appointments by preferred language

Expected result

A coordinated federal initiative would make existing protections operational: patients would receive understandable care, providers would have financing and practical support, and HHS would gain measurable evidence of implementation.

References

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